

Uveitis: phenotype, treatment burden and surgical readiness

Part 2 | Describe the phenotype, then test the hypothesis

A positive systemic test is not automatically the cause of the eye disease.

Evidence layer	Interpretation
Ocular pattern and time course	Sets the pretest probability
Exposure and systemic history	Makes particular tests relevant
Serology or immune sensitisation	May show exposure without proving ocular causation
Imaging or targeted sampling	Can resolve a competing mechanism

Infection can resemble noninfectious inflammation across several compartments. Syphilis deserves a low threshold for consideration in compatible unexplained disease. A positive TB immune test must be interpreted with the ocular phenotype, epidemiology and systemic assessment. Avoid treating an isolated laboratory result as a complete diagnostic argument.

Systemic associations are selected, not collected

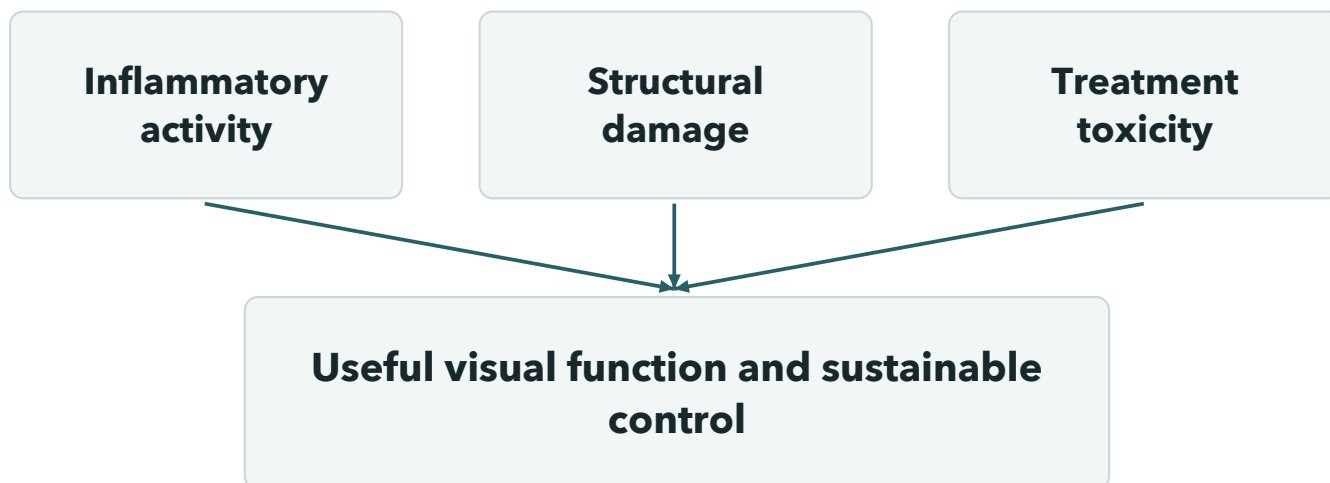
Normal screening results can coexist with a convincing clinical pattern.

Clinical clue	Focused assessment
Recurrent acute alternating anterior attacks	Explore spondyloarthritis-related symptoms
Granulomatous signs with posterior inflammation	Consider sarcoid and infectious mimics
Respiratory or systemic symptoms	Guide relevant imaging and specialist review
Discordant unilateral anatomy	Reconsider local disease rather than a systemic label

No single ACE result or plain chest image reliably excludes every sarcoid presentation. Tests should have a plausible consequence for diagnosis or treatment. A broad panel can generate incidental positives that distract from the phenotype. Revisit the hypothesis when new anatomy or an unexpected treatment response emerges.

Silent inflammation still creates treatment burden

The joints and the eyes can follow different trajectories.



Original balance: inflammatory control, structural complications and treatment toxicity must be followed together.

JIA-associated chronic anterior uveitis can be asymptomatic, so screening and follow-up cannot depend on pain. Track cells, synechiae, lens, pressure and macula alongside the steroid requirement. A child with quiet joints can still need active ocular treatment. Coordinate ophthalmic and rheumatologic decisions, including laboratory safety and adherence, rather than judging success from systemic symptoms alone.

Location determines what must be monitored

A quiet anterior chamber does not establish a quiet whole eye.

Compartment	Useful outcome
Vitreous and pars plana	Activity and peripheral complications
Retinal vessels	Leakage or occlusive consequences
Macula	Edema and structural visual limitation
Optic nerve and pressure	Damage from disease or its treatment

Intermediate inflammation may drive macular edema despite limited anterior activity. Imaging supports assessment but should answer a specific question. Distinguish active inflammation from residual structural damage when vision remains poor; escalating medication will not reverse every established scar or tractional change.

Choose a sustainable control strategy

Treatment escalation should have an explicit benefit and safety endpoint.

- Confirm active noninfectious disease and exclude relevant mimics.
- ↓
- Define visual threat and current steroid toxicity.
- ↓
- Choose a feasible steroid-sparing strategy with monitoring.
- ↓
- Reassess activity, structure, function and systemic adverse effects.

Consensus guidance supports systemic immunomodulation for persistent or severe disease and prevention of visual-threatening structural complications. Drug selection must account for phenotype, comorbidity and patient circumstances. Before cataract or capsule procedures, reconcile the inflammatory plan with macular and pressure risks; technical readiness is only one part of surgical readiness.